

Organizational and Individual Entity Data Brief

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NPD Scope Statement

What is an “organization” and an “individual” and what are the potential relationships between them is a convoluted problem that has been poorly reflected in NPPES data, and should be correctly reflected in the National Provider Directory (NDP).

FHIR itself expects to be able to model out the relationships between organizations and individual providers in a definitive way. However, the actual relationships between individual and organizational providers from CMS perspective are both incredibly detailed in some respects and in others it is blind to.

In order to understand what core data regarding organizations, individuals and the relationships between them, we have to consider a substantial amount of context from the existing PECOS and NPPES systems.

Background and Context

When registering to work with CMS in PECOS individuals and organizations have to provide context on:

- Who they are
- Where they intend to operate
- What their clinical specialties and qualifications are, including the right to prescribe and various supervision status like residency and mid-level practitioner constraints.
 - Different types of individual providers have specific details they must provide specific to their practice type/level
- What types of treatments they intend to perform or refer for treatment in each of the places they operate
- What is their relationship with certain other organizations
- Whether they have legal actions that impact their right to practice
- Whether they have the right to practice from the perspective of their state-level authority
- Any fundamental changes in their core status that might impact a previous enrollment
- How they would like to be paid (assignment)
- How the medical records of their patients will be maintained
- How they intend to bill CMS
- Whether they are generally accepting patients

FHIR standards

Unfortunately, the “Organization” FHIR object is overloaded.. For example, it could be a practice, a health system, a network operated by a health insurance carrier, a network leased by a health insurance carrier, and occasionally a solo practitioner And can potentially refer to may different kinds of organizations.

For individual providers there is Practitioner and PractitionerRole, with the latter referring to how an individual practices with regards to some clinical care delivery system and how they are contracted with a payer (i.e. directly through an Organization or through a Network)

Entity Structures in the data

Generally, for providers who are only in NPPES and not PECOS we cannot see as clearly what their corporate structures look like. CMS has the right to conduct extensive vetting for the providers that they allow to treat Medicare and Medicaid patients, and that extensive vetting results in an accurate picture of who owns, controls and runs the various healthcare providers that CMS collaborates with.

Entity types as expressed in NUCC taxonomies are orthogonal to organizational/individual status. So the provider types contradict the entity_type_code in NPPES. Group taxonomies are

used by individuals and individual taxonomies are used by organizations. There are limited specific use cases where that is handled according to the standard in the latter case.

Classes of individual Providers (as per 855I + NPPES)

Entity Bucket	Description	Impact on Billing	Impact on Payment	Impact on NPI Structure
Sole Proprietor (no Employer Identification Number (EIN))	The physician operates directly as an individual under their own legal name and SSN. No separate business entity exists under state law. A Sole Proprietor can assign payments to an organization, but not 100% of their income.	Bills Medicare directly under their personal SSN as the billing provider. All claims use the Type 1 NPI.	Payment goes directly to the individual (the same SSN). The person is the business.	Only one Type 1 NPI (individual). No Type 2 NPI exists.
Sole Proprietor (with EIN)	Still a natural person, but has obtained an EIN for tax or administrative purposes. No separate legal entity has been formed. A Sole Proprietor can assign payments to an organization, but not 100% of their income.	Bills as a sole proprietor using their EIN as TIN. Still enrolled as an individual (CMS-855I).	Payment is made to the individual's EIN bank account, but legally still the same person.	Still only one Type 1 NPI. The EIN doesn't create a new Type 2 NPI.

Individual Wholly Owns Org (LLC)	Individual Owning a Disregarded Entity (Single-Member LLC) Physician forms a single-member LLC that is disregarded for tax but legally distinct under state law.	Bills under the LLC's EIN and may use a Type 2 NPI for the entity; services can still reference the physician's Type 1 NPI as rendering.	Payment goes to the LLC's EIN (bank account of the entity).	Two NPIs: Type 1 for the person, Type 2 for the LLC. Both linked in PECOS via ownership.
Individual Wholly Owns Org (Not LLC)	Individual Owning a Corporation (e.g., PC, PA, S-Corp, C-Corp) Physician incorporates under state law; the corporation is a separate legal and tax entity.	Corporation bills as an organization using its own Type 2 NPI and EIN; physician's Type 1 NPI is listed as rendering provider.	Payment goes to the corporation's EIN/bank account. The corporation must file its own 855B (and often 855I for the physician).	Two NPIs: Type 1 (individual) + Type 2 (corporation). Treated as separate providers for enrollment.
Singleton Group	This is an individual provider paired perfectly with a single organizational NPI where the organization has a Tax number that is not linked to another organization. But the individual does not own 100% of the organization in question	Corporation bills as an organization using its own Type 2 NPI and EIN; physician's Type 1 NPI is listed as rendering provider.	Individual can choose to assign to the organization or not.. Depending.	Two NPIs: Type 1 (individual) + Type 2 (corporation). Treated as separate providers for enrollment.

Individual Connected to One Organization	Physician is contracted or employed by a single clinic, hospital, or group (not their own). There is no way to tell if what the contractual relationship is from PECOS. Nor is there any way to deduce if it is exclusive outside the CMS context. The organization is connected to other individual providers	Physician files a CMS-855I and reassigns billing to the organization's 855B enrollment. The organization submits claims.	In the outpatient context: Payment flows to the organization's EIN (via reassignment). The physician receives pay via employment/contract. The connection between institutional and individuals is listed in the practice location section B.	Type 1 NPI for the physician, Type 2 NPI for the organization; linked in PECOS via reassignment.
Individual Connected to Many Organizations	Physician has multiple active reassignments (e.g., hospital + telehealth + locums agency). Each organization may or may not be connected to other individual providers, but typically are.	Each organization bills under its own 855B. Each reassignment creates a separate billing relationship in PECOS.	Each organization is paid separately; the physician's personal payment depends on their private contracts.	Type 1 NPI used across all reassignments; multiple Type 2 NPIs from affiliated orgs appear in PECOS.
Locum Tenens / Substitute Physician	Temporary substitute for another enrolled physician (typically ≤ 60 days). Does not have their own separate billing	Uses the absent physician's NPI and PTAN for claims; no separate reassignment or enrollment is required. The HCPCS modifier for	Payment goes to the regular physician or their group , not to the locum directly.	Locum keeps their own Type 1 NPI for identity but it's not used for billing during the substitution.

enrollment for that
period.

locum tenens
services is Q6.

Why would anyone be a sole proprietor?

It is normally a business truism that you use corporations to isolate business risks from personal assets. However, most of the time medical malpractice “pierces the corporate veil”, which means that being a sole proprietor generally makes sense in the context of a particular states medical liability framework. So physicians might choose to be sole-proprietors when:

- They practice alone,
- They rely on malpractice insurance rather than entity shielding, and
- Their state’s medical practice act or board rules make incorporation optional or burdensome.
- Their state’s tax infrastructure makes having a corporate shell less ideal.

Organizational Entities (using 855A, other forms not considered)

Entity Type	Description (per 855A and CMS rules)	How this impacts billing	How this impacts payment	How this impacts NPI / structure
Single-facility institution (e.g., standalone hospital or SNF)	One legal business entity, one CCN and one PTAN, operates as a single institutional provider under one TIN.	Bills Medicare Part A directly under its PTAN/CCN.	Payment goes directly to the entity's EFT account.	One Type 2 NPI; subparts optional if internal departments bill separately.
Chain / Multi-facility organization	Parent company or "chain home office" with multiple separately enrolled facilities. Each site has its own CCN but common ownership.	Each facility bills under its own PTAN/CCN; chain may centralize billing operations.	Reimbursement and cost reports are facility-specific; home office costs are allocated.	Each facility with a separate billing stream has its own Type 2 NPI; the home office may also have a Type 2 NPI if it performs billable functions. Note that it is possible to have many addresses with a single NPI now so this is likely changing slowly.
Hospital with provider-based departments (PBDs)	Hospital includes off-campus or on-campus departments integrated under 42 CFR § 413.65.	Departments bill under the hospital's PTAN; claims identify PBD sites by	All payments flow to the hospital's main EFT account.	Hospital has one Type 2 NPI; departments are usually subparts with their own NPIs if they

		address/modifier.		transmit HIPAA transactions.
Hospital system with distinct part units (e.g., rehab or psych units)	Distinct part units within the same legal entity but with separate certification requirements. Sometimes these take the form of a Physician Office Building on campus. From what I've seen that involves a practice entity that is on campus, but may not be owned by the hospital ownership. Whether by contractual obligation or just convenience this may end up with the physician group referring patients to the hospital facilities for labs, imaging, pharmacy, or other services. Surgical practices may have specific contractual terms around use of the hospital's surgical facilities or post-surgical admissions. Such practices may also have non-hospital office locations. This can have implications for the patient in the form of facility fees, or plan designs which charge differently for in-hospital imaging, labs, etc.	Billed under the same CCN with specialty indicators; same PTAN.	Payment rates differ by unit type but flow through hospital's TIN.	Same Type 2 NPI; units may have separate subpart NPIs if required for HIPAA transactions.

Joint-venture institutional provider	Two or more owners (individuals or orgs) form a new entity that meets provider requirements.	The JV entity enrolls with its own 855A, CCN, PTAN, and bills under its own TIN.	Payment goes to the JV's account, not to the participants.	New Type 2 NPI specific to the JV; owners listed in Section 5 (organizations).
Government-owned provider	Federal, state, county, or tribal ownership (hospital district, city-county, etc.).	Bills under government TIN; often exempt from application fee.	Payment made to the government entity's account.	Type 2 NPI using government's LBN/TIN; ownership attestation required.
Private equity / holding-company structure	Ownership is through LLCs or PE funds; may involve multiple tiers of indirect control.	Billing remains with the operating entity's PTAN; CMS must see full ownership chain in Section 5.	Payment to the operating provider, though investors may ultimately own the proceeds.	Each operating entity has its own Type 2 NPI; holding companies are reported but don't get NPIs unless they furnish care.
Non-profit / religious / charitable corporation	501(c)(3) organization governed by a board; may run one or more facilities.	Bills as organizational provider; CHOWs require board approval.	Payment to the corporate entity; may pool for multiple facilities.	Typically a Type 2 NPI for each facility; "board of trustees" listed as owner/controller in Section 5. The same "farway NPI" effect sometimes applies here.

Consolidated organization (merger forming new entity)	Two or more previously enrolled entities dissolve and form a new business under new TIN and CCN.	Old PTANs/CCNs retire; new application and certification required.	Payments flow only after new CCN activation.	New Type 2 NPI must match new LBN/TIN.
Change of ownership (CHOW)	Existing facility sold; buyer may assume or decline prior provider agreement.	If accepted, CCN transfers; if declined, new enrollment required.	If accepted, payment continues under existing agreement; otherwise pauses pending approval.	NPI may stay if LBN/TIN match NPPES; new entity may obtain new Type 2 NPI.

Status Transitions

As a given organization is sold or has partial ownership changes, or other changes occur that would cause an organization to move from one category to another in the table above. They generally have to submit new enrollment forms to CMS upon those changes occurring.

Organizational NPIs

In the U.S. healthcare system, an **organization's legal identity** is primarily defined by its **Employer Identification Number (EIN)**, while its **billing and transactional identity** under HIPAA is expressed through one or more **organizational NPIs (Type 2)**. A single EIN can encompass many NPIs, each corresponding to a distinct billing flow, facility, or service type that participates in HIPAA-covered electronic transactions. These NPIs may be arranged hierarchically—where one organizational NPI is designated as the parent of a subpart NPI representing a distinct department, location, or billing stream. This layered NPI structure enables compliance with payer and transaction requirements while reflecting the operational complexity of large healthcare entities, such as hospital systems or nationwide pharmacy chains, which may require unique NPIs for each physical location or service line.

While the legal identity is associated clearly with an EIN, the "operational identity" can vary greatly. EINs reflect the legal entities through which business transactions are made. But a corporate structure can be made up of multiple EINs in whatever way is advantageous to the owners and corporations involved. For this reason, depending on the type of entity, the enrollment form also requests information about who is responsible for the organization in the end.

There is **no uniform or enforceable national rule** dictating when an organization must—or must not—create additional NPIs. CMS and NPPES guidance advises that proliferation of organizational NPIs should occur **only when justified by functional billing distinctions**, but enforcement is effectively self-attested. As a result, NPI enumeration patterns vary widely across provider types and corporate structures. Some organizations consolidate billing under a single NPI, while others, like Walgreens or CVS, properly maintain thousands for individual sites. Certain provider categories—such as home health agencies or laboratories—face **explicit regulatory requirements** to enumerate subparts separately, further fragmenting the landscape. The result is a flexible but inconsistent system in which the EIN anchors legal accountability, while NPIs reflect the granular operational interfaces through which healthcare organizations actually transact with payers.

Specifically relating to location: the organizational NPI used in billing is not always representative of the physical location where services were provided. It is quite common for many physical entities to bill through one (faraway) NPI.

Individual Non-Clinical Roles

Then we have the fundamental relationships that individuals can have with both individuals and organizations contracted with the government. These include:

- **Ownership** → who can profit or lose.
- **Managing control** → who runs daily operations.
- **Authorization** → who binds the entity to CMS.
- **Delegation** → who communicates updates.
- **Reassignment** → who receives payment.

The reassignment construct is quite specific to Medicare. Medicare's structure of physicians accepting Medicare appointment and then being conferred right to payment directly from CMS for services rendered to Medicare beneficiaries is relatively unique in payer-provider payment arrangements.

In most other payment arrangements the insurer/payer contracts with some legal entity representing the physician. This contract (nominally) requires that legal entity to accept patients holding that insurance and to accept the payment rates and rules, covered services negotiated for those patients.

For instance, in other arrangements there is no "reassignment" from the physician to the group because the group already holds the right to collect reimbursements. Providers are paid

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according to their contracts with those groups.

In some cases, a contract between a payer and provider organization has named practitioners, and in other cases, the contract is abstracted away from any particular practitioner (generally, in cases where credentialing and enrollment are 'delegated' to the provider organization).

On NPI Subparts

NPI subparts are organizational NPIs that list a parent Organizational NPI as a parent.

The short answer is that if a component of the same organization and facility needs to conduct separate HIPAA-covered transactions, then it may need a sub-part NPI in order to differentiate the billing streams. Often it is not CMS that requires a provider to separate their billing streams in this manner, but a different payer.

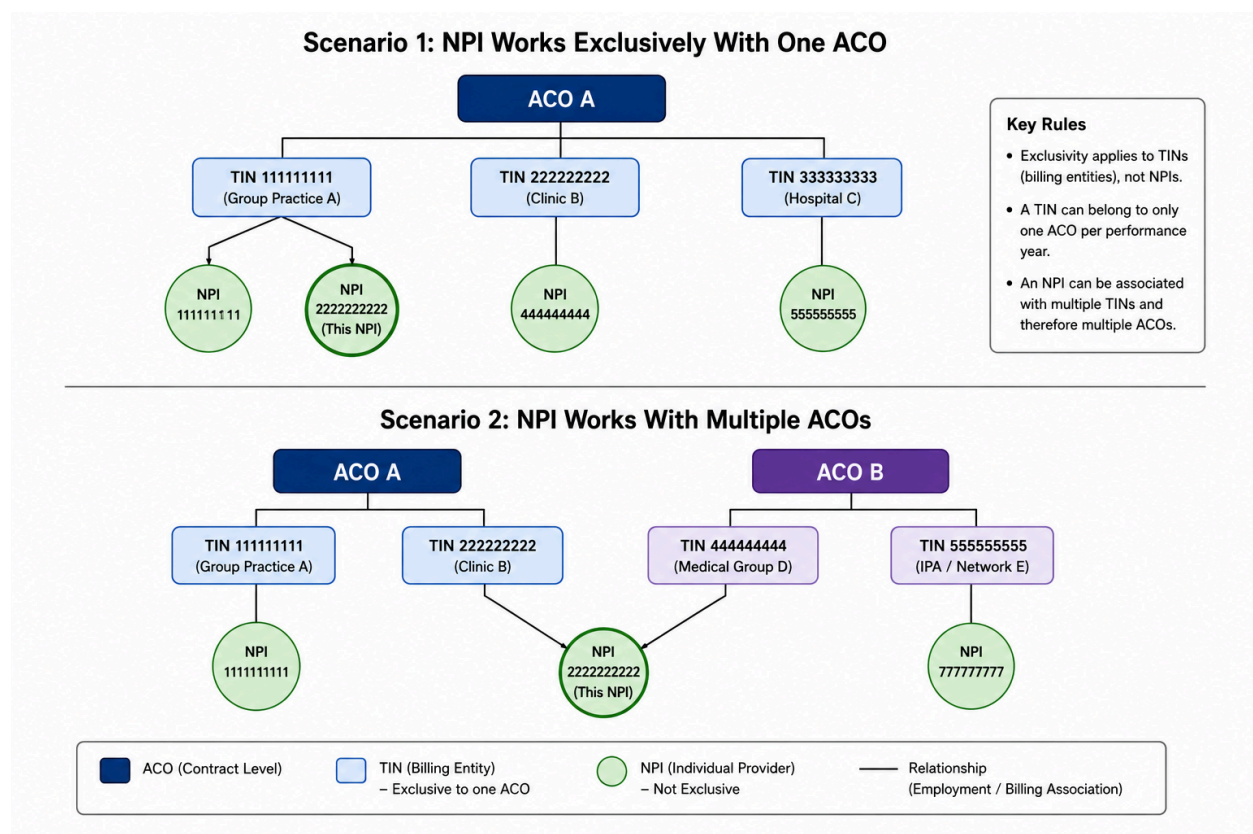
CMS issued the following guidance on this topic:

- [Guidance on Subpart Determination for Medicate Organization Providers who are Covered Entities under HIPAA March 2006](#)
- [The Who, What, When, Why & How of NPI: Information for Health Care Providers August 2006](#)
- [For healthcare providers who are organizations July 2007](#)
- [Guidance on how other plans can or cannot require additional sub-part NPIs](#)

Accountable Care Organizations

Accountable Care Organizations (ACOs) add a new **layer of organizational order** to the healthcare system that sits *above* individual providers and traditional healthcare organizations. Whereas physicians and hospitals bill Medicare as discrete entities through their Tax Identification Numbers (TINs), ACOs form networks of these existing participants under a shared accountability contract with CMS. They do not replace fee-for-service billing or enrollment; rather, they overlay a system of collective responsibility for the cost and quality of care across attributed patient populations. This creates a superstructure of governance, data sharing, and incentive alignment that unites disparate providers under common performance metrics, even though they remain operationally and financially independent billing entities.

Within this structure, **exclusivity applies to TINs, not NPIs**. CMS attributes Medicare beneficiaries to ACOs based on the primary care services billed under specific TINs, which represent the legal and financial entities responsible for claims. A TIN that furnishes such services can belong to only one ACO for a given performance year, ensuring each beneficiary is assigned to a single accountable entity. However, an individual provider—identified by their personal NPI—may practice under multiple TINs, and thus participate in more than one ACO through those separate billing relationships. This distinction allows a physician's professional identity (NPI) to span multiple organizational contexts, while preserving the one-to-one alignment required for Medicare attribution and shared savings calculations at the TIN level.



At one time, ACO contracts were generally arranged as bets CMS would make with primary care providers against specialists and hospitals. The idea was that an ACO, by providing excellent primary care, could reduce specialty and hospital costs that were otherwise uncontrolled. By keeping patients healthy, primary care could lower the overall cost of the healthcare system.

However, over time, healthcare systems began launching their own ACOs as a kind of hedge against the program.

For example:

- Suppose you are System A.
- Instead of allowing an external ACO (ACO X) to manage your attributed patients,
- And instead of risking that an outside ACO would steer patients away from your hospital.
- You create your own ACO.

The presumption is that when those patients eventually need hospitalization, they will stay within your system. In this way, System A creates ACO A that it owns and controls.

There were also cases where specialty groups created ACOs for similar reasons.

This created new opportunities for efficiency, such as:

- Coordinated management of chronic conditions (e.g., diabetes).
- Specialty-led models focused on improving outcomes and reducing complications.
- Financial arrangements that reward keeping patients healthy rather than increasing utilization.

Overall, the model evolved from a primary care–driven cost-control strategy into a broader set of organizational and financial strategies used by healthcare systems and specialty groups.

Often ACOs grow in the Medicare Advantage Plans

As ACOs matured, many discovered that the shared savings model of MSSP limited their upside while leaving key variables—like patient attribution and network leakage—only partially under their control. This led to a natural progression toward Medicare Advantage (Part C), where organizations can capture the full premium, directly manage enrollment, and design networks and benefits to steer care. Economically, the shift reflects a move from participating in savings generated within fee-for-service Medicare to owning the total cost and revenue stream associated with a defined population.

This transition has driven consolidation and vertical integration, as success in Medicare Advantage requires scale, actuarial capability, and tightly managed provider networks. Large ACOs increasingly evolve into payer-provider hybrids or partner closely with insurers, using the infrastructure they built under MSSP—care management, data systems, and provider

alignment—as a foundation. In effect, ACOs have become a stepping stone from care coordination entities into fully risk-bearing organizations that function as insurance plans.

Healthcare Systems

At the top level, the U.S. healthcare system is organized as a **federation of legally distinct corporate entities** that align through ownership, branding, and contractual relationships rather than a single unified structure. Each large healthcare system typically operates as a **cluster of state-specific corporations**, each with its own **Employer Identification Number (EIN)**, because licensure, insurance contracting, and regulatory reporting occur primarily at the state level. These entities appear in **PECOS and CMS enrollment records** as separate organizations, often linked through ownership hierarchies that reveal their common parent or management structure. The resulting landscape is not a single monolithic corporation, but a **graph of interrelated legal entities** that collectively form a recognizable healthcare “system.” What unites them in practice is shared governance, a centralized board or holding company, common IT and HR infrastructure, and frequently, a unified web domain and brand—though in some cases, each state-level entity maintains its own branding and local identity.

Thus, a “healthcare system” is best understood not as one business or one hospital, but as a **multi-EIN ecosystem** that spans hospitals, clinics, and ancillary organizations under a shared strategic, financial, or brand umbrella. Whether a system is recognized as unified depends less on strict ownership thresholds and more on how it **coordinates patient care, integrates data systems, and presents itself to the public and payers**. Some systems maintain consistent national branding (e.g., “Ascension” or “HCA Healthcare”), while others deliberately present separate regional identities despite common ownership to align with local market or regulatory conditions. In essence, the healthcare “system” is a conceptual layer that sits **above the legal and tax structures** of individual entities—binding them together operationally and reputationally, even as compliance and contracting realities keep them fragmented at the state and EIN level.

CMS Certification Number

The **CMS Certification Number (CCN)** is the institutional identifier that CMS uses to recognize and track Medicare-certified facilities for reimbursement, compliance, and quality oversight. It differs from the **PTAN (Provider Transaction Access Number)**, which is assigned by a Medicare Administrative Contractor (MAC) for billing and payment purposes. While a provider may have multiple PTANs—for example, one per practice location or enrollment record—the CCN represents the legally certified entity that CMS surveys and regulates. Large volumes of reporting flow through the CCN, including mandatory data to CMS, the American Hospital Association (AHA), and national quality and safety programs. A single CCN may cover several organizational NPIs, tying together the billing and reporting flows for multiple facilities or departments under one certification contract. The CCN itself encodes intelligence in its structure: prefixes identify the state and provider type, and the numbering scheme links the organization to a specific CMS certification and reimbursement relationship.

Special Hospital Types

A **swing-bed hospital** is typically a small or rural hospital that provides both acute care and **Skilled Nursing Facility (SNF)** services under one certification. The swing-bed program, established under the 1980 Omnibus Reconciliation Act, enables hospitals to convert inpatient beds to SNF beds as needed without transferring patients to a separate facility. These hospitals must meet both acute and SNF regulatory requirements and maintain distinct cost reporting for each service category under the same CCN. This dual-use model helps maintain access to post-acute care in rural areas and ensures more efficient use of limited inpatient resources.

Several types of institutions are **exempt from the standard Inpatient Prospective Payment System (IPPS)** due to their unique missions or patient populations. These include **cancer hospitals, children's hospitals, rehabilitation hospitals, and long-term care hospitals**. Such facilities are reimbursed through specialized payment methodologies that better reflect their case mix and resource intensity. National Cancer Institute–designated hospitals like MD Anderson and Memorial Sloan Kettering, for instance, operate under this exemption because their complex oncology focus and research obligations make them incompatible with standard IPPS rate structures.

A distinct policy applies to **physician-owned hospitals**, which have been subject to restrictions since the Affordable Care Act (Section 6001) in 2010. Facilities that were already operating and participating in Medicare before that date were allowed to continue doing so, under strict limits on ownership changes and expansion. However, **physician-owned hospitals established after 2010 are explicitly excluded from Medicare participation**—they are not covered under this continuation allowance and therefore cannot bill Medicare for services. This policy was designed to prevent conflicts of interest and limit the expansion of physician self-referral models while preserving continuity for previously participating institutions.

Finally, **psychiatric hospitals** form a distinct provider category regulated under their own participation conditions (42 CFR Part 482 Subpart E) and reimbursed through the **Inpatient Psychiatric Facility Prospective Payment System (IPF PPS)**. This system applies per-diem reimbursement rates adjusted for patient characteristics, facility type, and length of stay. Psychiatric hospitals must provide continuous nursing care, employ a psychiatrist as medical director, and maintain structured treatment programs to retain certification. Their separate classification acknowledges the specialized nature of mental health care and ensures quality and compliance oversight suited to behavioral health rather than general medicine.

Federal health systems occupy a unique position within the Medicare framework. Historically, **Indian Health Service (IHS)** facilities and **Veterans Affairs (VA)** hospitals operated entirely outside the Medicare payment system, as they were federally funded and served specific beneficiary populations. Over time, both IHS and VA systems have chosen to **cross-participate with Medicare** under limited arrangements to enable reimbursement for care provided to Medicare beneficiaries under certain conditions—such as services delivered in non-federal settings, joint ventures, or referrals between systems. These collaborations allow data and funding to flow between federal and civilian care systems while maintaining distinct regulatory

oversight. In contrast, **military treatment facilities (MTFs)** operated by the Department of Defense remain largely outside the Medicare billing infrastructure. They serve active-duty personnel and dependents under the TRICARE program rather than through Medicare reimbursement. As a result, while IHS and VA hospitals have developed points of interoperability with Medicare, **DoD hospitals continue to function as closed, non-Medicare systems** within the broader federal healthcare landscape.

NPI not strictly “organizational” vs “individual”

The **NUCC taxonomy code system** defines provider types in a dual structure—one branch for **individuals** and another for **organizations**—but in practice, the distinction is often blurred. For example, the taxonomy tree includes “**group practice**” as an individual type, even though it describes a collection of practitioners, not a single person. Typically, an **organizational NPI (Type 2)** should select a taxonomy from the organizational branch, while a **personal NPI (Type 1)** should select one from the individual branch. However, cross-assignments occur regularly, sometimes for legitimate hybrid cases (like solo practices using both types of identifiers) but often as strange billing patterns. This misalignment means that the NUCC taxonomy hierarchy—the conceptual classification of provider types—is **not well synchronized** with how the NPI registry (NPPES) distinguishes individual versus organizational entities. The result is a taxonomy framework that theoretically enforces a clean division but, in real-world data, frequently shows overlap and confusion at the edges.

This complexity is compounded by how NPIs are represented on **medical claims forms (X12 and paper)**, where the fields are rigidly typed for either personal or organizational identifiers. Institutional claims (UB-04/837I) include explicit fields for **attending, operating, or rendering provider NPIs**, which should be Type 1 (individual), while the billing or pay-to provider fields should carry a Type 2 (organizational) NPI. Yet enforcement of these distinctions is inconsistent across payers and clearinghouses. As a result, **cross-field misuse is common**—individual NPIs sometimes appear as billing providers, and organizational NPIs are sometimes entered where a rendering provider should be.

This produces a **three-way matrix of conflation** between (1) the entity-type code in NPPES, (2) the NUCC taxonomy’s intended classification of that provider, and (3) the positional rules for NPIs on billing forms. In theory, each axis should align neatly; in practice, all three systems—taxonomy, entity typing, and claim-form field design—interact imperfectly, creating one of the most persistent sources of structural ambiguity in healthcare provider data.

Due to the limitations described above, information in NPPES about addresses is highly confounded and unreliable. Although data on “practice location” is nominally collected within NPPES, these addresses rarely provide an accurate representation of the locations where a patient could book any appointment to see that provider. Even when the addresses with NPPES are in fact intended to be a representation of the physical location, they often do not provide a good geographic indication of where the provider’s practice location actually is and how to get there. However, since NPPES is the only “registry” of physicians, the data on organizations is often pulled into public websites and consumer-facing applications, creating a great deal of noise around how to locate a provider’s physical practice site.

Open Questions

I cannot get a definitive answer on the question: “Is it possible to bill Medicare Advantage or Medicaid Managed Care Organizations (MCO) and not be registered in PECOS?” Apparently this is possible... but uncommon because of a requirement to make sure that a “The provider is “eligible to receive payment under Medicare” (i.e., not excluded or revoked) as per 42 CFR §422.204(b)(3). The simplest way to do that is just make sure they are in PECOS.

Example NPI to research:

ein 364214409
type 2 npi 1194812693
type 1 npi 1467441519

I am unclear about the boundary between hospitals and rehab facilities. I have personally seen separate bills from rehab facilities that by appearances are processed by different billers. They may lead back to the same legal entity but it's not obvious that they must. Also, it's unclear to me when rehab stays would be part of a DRG as opposed to distinct.

Data Strategy Sequence

Technology Changes

Recommended Policy Changes:

Resources: